Redcliffe labs

Healthy India Ki Trusted Lab

smart Health Report

An Insightful Health Analytics Report for Easier Understanding

Prepared For

Ms Sonal Mittal

F 23

Name
Ms Sonal Mittal

Gender
F

Patient ID
15214055

Age
23

Your Health Summary

Understand Your Health At A Glance
Your Personalized Health Summary is Now Available.

View Detailed Summary on our App

Your Health at a Glance – A Personalized Journey

Report Sections

1. **Body Summary**
   A visual snapshot of your overall health, simple and easy to understand
2. **Quick Health Highlights**
   Your health scores and a single view of all abnormal results for quick attention
3. **Lab Report Overview**
   Understand at a glance which tests are normal and which are abnormal
4. **Personalized Health Advisory**
   Actionable insights and expert guidance tailored just for you
5. **Doctor's Reference Report**
   Complete lab results with interpretations to share with your healthcare provider

How to Read This Report

This comprehensive health report provides detailed insights into your test results. Each section offers different perspectives on your health status, from visual summaries to detailed analysis and personalized recommendations.

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

SMART HEALTH REPORT

ISO 9001 :2015

Redcliffe labs

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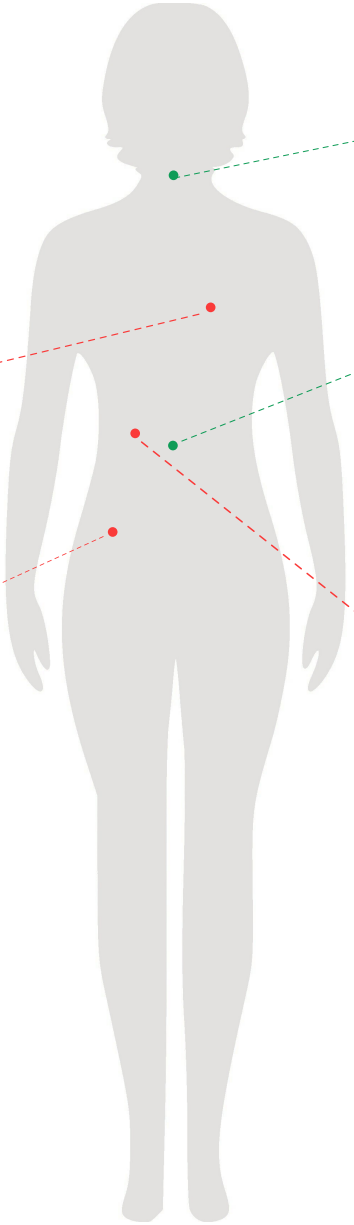
Health Summary



BLOOD COUNTS

Test Name	Result
Lymphocytes	45
Lymphocytes.	3.69

Please Watchout

CARDIAC PROFILE

Test Name	Result
HDL Cholesterol	37.1
HDL/ LDL Ratio	0.43
LDL/HDL Ratio	2.31

Please Watchout

KIDNEY PROFILE

Test Name	Result
Blood Urea	15.7
Deposit	Present

Please Watchout

ELECTROLYTES

Test Name	Result
Chloride	108.2

Please Watchout

VITAMIN PROFILE

Test Name	Result
Vitamin - B12	166
Vitamin D 25 - Hydroxy	11.9

Please Watchout

THYROID PROFILE

Everything looks good

DIABETES MONITORING

Everything looks good

LIVER PROFILE

Test Name	Result
Bilirubin Direct	0.23

Please Watchout

ANEMIA STUDIES

Everything looks good

MINERAL PROFILE

Everything looks good

Note: This section offers a quick snapshot of parameters. For detailed analysis and clinical interpretation, please refer to the following pages

View Report on App

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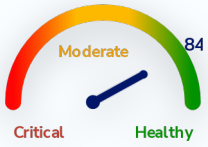
Quick Health Summary

Personal Insights - Score

84 (Excellent)

Your health assessment indicates excellent scores in critical areas such as Diabetes, Anemia, and Allergy, reflecting strong management. However, there is a notable concern with Vitamins and Minerals, suggesting a need for dietary adjustments. Overall, your health is stable, but consider lifestyle changes to enhance your well-being.



Summary of Key Health Indicators

Total Parameters Tested	Abnormal Results
96	17

Health Status by Body System

Profile	Abnormal / Total	Key Results
Cardiac Profile	3 / 10	HDL Cholesterol: 37.1 mg/dL (Normal: 40–80 mg/dL) HDL/ LDL Ratio: 0.43 Ratio (Normal: 0.5–3.0 Ratio) LDL/HDL Ratio: 2.31 % (Normal: 2.5–3.5 %)
Blood Clotting	3 / 5	Mean Platelet Volume (MPV): 12.6 fL (Normal: 9.3–12.1 fL) PDW: 25.9 fL (Normal: 8.3–25 fL) P-LCR: 56.1 % (Normal: 18–50 %)
Urinalysis	3 / 17	Transparency: Hazy (Normal: –) Blood: Positive(+) (Normal: –) Red blood Cells: 10-12 /hpf (Normal: – /hpf)
Blood Counts	2 / 14	Lymphocytes: 45 % (Normal: 20–40 %) Lymphocytes.: 3.69 10^3/µl (Normal: 1–3 10^3/µl)
Kidney Profile	2 / 13	Blood Urea: 15.7 mg/dL (Normal: 16.6–48.5 mg/dL) Deposit: Present (Normal: –)
Vitamin Profile	2 / 2	Vitamin - B12: 166 pg/mL (Normal: 180–914 pg/mL) Vitamin D 25 - Hydroxy: 11.9 ng/mL (Normal: 30–100 ng/mL)
Liver Profile	1 / 12	Bilirubin Direct: 0.23 mg/dL (Normal: 0–0.2 mg/dL)

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Profile	Abnormal / Total	Key Results
Electrolytes	1 / 3	● Chloride: 108.2 mmol/L (Normal: 98–107 mmol/L)
Diabetes Monitoring	0 / 3	All Normal
Anemia Studies	0 / 11	All Normal
Inflammation	0 / 1	All Normal
Mineral Profile	0 / 1	All Normal
Thyroid Profile	0 / 3	All Normal

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Report Summary

Normal | Abnormal | No color - Reference range not available

DIABETES MONITORING

Test Name	Result unit	Range
Glycosylated Hemoglobin (HbA1c)	5.2 %	< 5.6
Estimated Average Glucose	102.54 mg/dL	
Glucose Fasting	78.6 mg/dL	70-100

CARDIAC PROFILE

Test Name	Result unit	Range
HIGHLY SENSITIVE C-REACTIVE PROTEIN (hs-CRP)	0.3 mg/L	< 1
Total Cholesterol	133 mg/dL	< 200
Triglycerides	51.5 mg/dL	< 150
HDL Cholesterol	**37.1** mg/dL	40-80
Non HDL Cholesterol	95.9 mg/dL	< 130
LDL Cholesterol	85.6 mg/dL	30-100
V.L.D.L Cholesterol	10.3 mg/dL	< 30
Chol/HDL Ratio	3.58 Ratio	3-5
HDL/ LDL Ratio	**0.43** Ratio	0.5-3
LDL/HDL Ratio	**2.31** %	2.5-3.5

ANEMIA STUDIES

Test Name	Result unit	Range
Hemoglobin	12.4 g/dL	12-15
PCV	36.6 %	36-46
MCV	86.4 fl	83-101
MCH	29.3 pg	27-32
MCHC	33.9 g/dL	31.5-34.5
RDW (CV)	12.1 %	11.6-14
RDW-SD	43.7 fl	35.1-43.9
Iron	78.8 µg/dL	33-193
TIBC,(Total Iron Binding Capacity)	302.8 µg/dL	250-450
UIBC	224 µg/dL	135-392
Transferrin Saturation	26.02 %	16-45

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BLOOD COUNTS

Test Name	Result unit	Range
RBC Count	4.2 10^6/µl	3.8-4.8
TLC	8.2 10^3/µl	4-10
Neutrophils	46.6 %	40-80
Lymphocytes	**45** %	20-40
Monocytes	3.8 %	2-10
Eosinophils	3.6 %	1-6
Basophils	1 %	< 2
Neutrophils.	3.82 10^3/µl	2-7
Lymphocytes.	**3.69** 10^3/µl	1-3
Monocytes.	0.31 10^3/µl	0.2-1
Eosinophils.	0.3 10^3/µl	0.02-0.5
Basophils.	0.08 10^3/µl	< 0.5
Platelet Count	169 10^3/µl	150-410
Mentzer Index	20.57 %	

BLOOD CLOTTING

Test Name	Result unit	Range
Mean Platelet Volume (MPV)	**12.6** fL	9.3-12.1
PCT	0.2 %	0.17-0.32
PDW	**25.9** fL	8.3-25
P-LCR	**56.1** %	18-50
P-LCC	95 10^9/L	44-140

INFLAMMATION

Test Name	Result unit	Range
ESR - Erythrocyte Sedimentation Rate	7 mm/hr	< 12

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Normal | Abnormal | No color - Reference range not available

LIVER PROFILE

Test Name	Result unit	Range
Bilirubin Total	0.4 mg/dL	< 1.2
Bilirubin Direct	**0.23** mg/dL	< 0.2
Bilirubin Indirect	0.17 mg/dL	< 1
SGOT/AST	16.8 U/L	5-40
SGPT/ALT	15.3 U/L	< 41
SGOT/SGPT Ratio	1.1 %	
Alkaline Phosphatase	64.9 U/L	35-104
Total Protein	6.6 g/dL	6-7.8
Albumin	4 g/dL	3.5-5.2
Globulin	2.6 g/dL	2.3-3.5
Albumin :Globulin Ratio	1.54	< 2.1
Gamma Glutamyl Transferase (GGT)	9.5 U/L	< 40

KIDNEY PROFILE

Test Name	Result unit	Range
Blood Urea	**15.7** mg/dL	16.6-48.5
Bun	7.34 mg/dL	6-20
Creatinine	0.5 mg/dL	0.5-0.9
eGFR (CKD-EPI)	135.06 ml/min/1.73 sq m	
Bun/Creatinine Ratio	14.68	12-20
Urea / Creatinine Ratio	31.4	25.68-42.8
Uric Acid	4.1 mg/dL	2.4-5.7
Calcium Serum	9.2 mg/dL	8.6-10
Colour	Yellow	
Deposit	**Present**	
Urine Glucose (sugar)	Negative	
Yeast Cells	Absent	
Amorphous deposits	Absent	

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MINERAL PROFILE

Test Name	Result unit	Range
Phosphorus	4.3 mg/dL	2.5-4.5

ELECTROLYTE PROFILE

Test Name	Result unit	Range
Sodium	142.6 mmol/L	136-145
Potassium	4.6 mmol/L	3.5-5.1
Chloride	**108.2** mmol/L	98-107

VITAMIN PROFILE

Test Name	Result unit	Range
Vitamin - B12	**166** pg/mL	180-914
Vitamin D 25 - Hydroxy	**11.9** ng/mL	30-100

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THYROID PROFILE

Test Name	Result unit	Range
Triiodothyronine (T3)	1.1 ng/mL	0.71-2.01
Total Thyroxine (T4)	7 µg/dL	5.1-14.1
Thyroid Stimulating Hormone (Ultrasensitive)	2.2 µIU/mL	0.27-4.2

URINALYSIS

Test Name	Result unit	Range
Volume	20 mL	
Transparency	**Hazy**	
Reaction (pH)	6.0	
Specific Gravity	1.010	
Urine Protein (Albumin)	Negative	
Urine Ketones (Acetone)	Negative	
Blood	**Positive(+)**	
Leucocyte esterase	Negative	
Bilirubin Urine	Negative	
Nitrite	Negative	
Urobilinogen	Normal	
Pus Cells (WBCs)	2-4 /hpf	
Epithelial Cells	1-2 /hpf	
Red blood Cells	**10-12** /hpf	
Crystals	Absent	
Cast	Absent	
Bacteria	Absent	

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Health Advisory

Normal (N) Low (L) High (H)

Cardiac Profile

Most people believe they are safe from heart diseases, but in reality, heart diseases are the leading cause of death in the world. There are many different forms of heart disease. Narrowing or blockage of the coronary arteries is the most common cause of heart disease, which are the vessels that supply blood to the heart. This is called coronary artery disease and it occurs slowly over time. It is the main cause of heart attacks.

HDL Cholesterol: 37.1 mg/dL — LOW

LOW	NORMAL	HIGH
< 40	40-80	> 80

You: 37.1

Vitamins Profile

Vitamins are considered essential nutrients because they perform hundreds of roles in your body. They help maintain bones, heal wounds, and strengthen your immune system. They also convert food into energy, and repair cellular damage

Vitamin - B12: 166 pg/mL — LOW

LOW	NORMAL	HIGH
< 180	180-914	> 914

You: 166

Vitamin D 25 - Hydroxy: 11.9 ng/mL — LOW

LOW	NORMAL	HIGH
< 30	30-100	> 100

You: 11.9

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LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29157878
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 01:53 PM.

Fit India Full Body Checkup With Vitamin Screening with Free Heart Test (HsCRP)

Complete Blood Count (CBC)

Test Description	Value(s)	Unit(s)	Reference Range
RBC Parameters			
Hemoglobin *Cyanide Free Spectrophotometry*	12.4	g/dL	12.0 - 15.0
RBC Count *Electrical impedance*	4.2	10^6/µl	3.8 - 4.8
PCV *Calculated*	36.6	%	36 - 46
MCV *Calculated*	86.4	fl	83 - 101
MCH *Calculated*	29.3	pg	27 - 32
MCHC *Calculated*	33.9	g/dL	31.5 - 34.5
RDW (CV) *Calculated*	12.1	%	11.6 - 14.0
RDW-SD *Calculated*	43.7	fl	35.1 - 43.9
WBC Parameters			
TLC *Electrical impedance and microscopy*	8.2	10^3/µl	4 - 10
Differential Leucocyte Count			
Neutrophils *Leishman Staining & Microscopy And Flow Cytom*	46.6	%	40-80
Lymphocytes *Leishman Staining & Microscopy And Flow Cytom*	**45**	%	20-40
Monocytes *Leishman Staining & Microscopy And Flow Cytom*	3.8	%	2-10
Eosinophils *Leishman Staining & Microscopy And Flow Cytom*	3.6	%	1-6
Basophils *Leishman Staining & Microscopy And Flow Cytom*	1	%	<2
Absolute Leukocyte Counts			
Neutrophils. *Calculated*	3.82	10^3/µl	2 - 7
Lymphocytes. *Calculated*	**3.69**	10^3/µl	1 - 3
Monocytes. *Calculated*	0.31	10^3/µl	0.2 - 1.0
Eosinophils. *Calculated*	0.3	10^3/µl	0.02 - 0.5
Basophils.	0.08	10^3/µl	0.02 - 0.5

Dr. Preeti Mathur
MBBS, MD (Path)
Consultant Pathologist

Patient NAME	**: Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29157878
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 01:53 PM.

Test Description	Value(s)	Unit(s)	Reference Range
Calculated			
Platelet Parameters			
Platelet Count *Electrical impedance and microscopy*	169	10^3/µl	150 - 410
Mean Platelet Volume (MPV) *Calculated*	**12.6**	fL	9.3 - 12.1
PCT *Calculated*	0.2	%	0.17 - 0.32
PDW *Calculated*	**25.9**	fL	8.3 - 25.0
P-LCR *Calculated*	**56.1**	%	18 - 50
P-LCC *Calculated*	95	10^9/L	44 - 140
Mentzer Index *Calculated*	20.57	%	> 13

Interpretation:
CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

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Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29157878
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 03:52 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate *MODIFIED WESTERGREN*	7	mm/hr	0 - 12

Interpretation:
ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis ,systemic vasculitis, anemia, multiple myeloma , etc. Low levels are typically seen in congestive heart failure, polycythemia ,sickle cell anemia, hypo fibrinogenemia , etc.

Reference- Dacie and lewis practical hematology

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LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29157878
Referred BY	: Self	Sample Type	: Whole blood EDTA
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 02:25 PM.

Test Description	Value(s)	Unit(s)	Reference Range

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) *HPLC*	5.2	%	< 5.7
Estimated Average Glucose *Calculated*	102.54	mg/dL	Refer Table Below

Interpretation:
Interpretation For HbA1c% As per American Diabetes Association (ADA)

Reference Group	HbA1c in %
Non diabetic adults >=18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	>= 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:
1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :
HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

Preeti

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Patient NAME	: **Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 30600433
Referred BY	: Self	Sample Type	: FLUORIDE F
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 01:43 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Glucose Fasting

Glucose Fasting *Hexokinase*	78.6	mg/dL	70 - 100

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association

Comment :
Blood glucose determinations in commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy insulinoma, or various liver diseases.

Note
1.The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL or a random / 2 hour plasma glucose value of > or = 200 mg/dL with symptoms of diabetes mellitus.
2.Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

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Patient NAME	: **Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29930269
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 02:28 PM.

Liver Function Test (LFT)

Test Description	Value(s)	Unit(s)	Reference Range
Bilirubin Total *Diazonium Salt*	0.4	mg/dL	0 - 1.2
Bilirubin Direct *Diazo Reaction*	**0.23**	mg/dL	0 - 0.20
Bilirubin Indirect *Calculation (T Bil - D Bil)*	0.17	mg/dL	0.1 - 1.0
SGOT/AST *IFCC with P5P*	16.8	U/L	up to 40
SGPT/ALT *IFCC with P5P*	15.3	U/L	up to 41
SGOT/SGPT Ratio *Calculated*	1.1	-	-
Alkaline Phosphatase *Para-Nitrophenyl Phosphate*	64.9	U/L	35 - 104
Total Protein *Biuret*	6.6	g/dL	6.0 - 7.8
Albumin *BCG*	4	g/dL	3.5 - 5.2
Globulin *Calculation (T.P - Albumin)*	2.6	g/dL	2.3 - 3.5
Albumin :Globulin Ratio *Calculation (Albumin/Globulin)*	1.54	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) *L-Gamma glutamyl-3-Carboxy-4- Nitroanilidesubstrat*	9.5	U/L	5-40

Interpretation:
The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

1. **AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
2. **ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
3. **Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

1. **Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
2. **High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
3. **Low albumin:** Caused by poor diet, kidney, or liver disease.
4. **High albumin:** Usually due to severe dehydration.

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Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 02:28 PM.

Kidney Function Test (KFT)

Test Description	Value(s)	Unit(s)	Reference Range
Blood Urea *Urease*	**15.7**	mg/dL	16.6 - 48.5
Bun *Calculated*	7.34	mg/dL	6 - 20
Creatinine *Jaffe*	0.5	mg/dL	0.50 - 0.90
eGFR (CKD-EPI) *Calculated*	135.06	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio *Calculated*	14.68		12 - 20
Urea / Creatinine Ratio *Calculated*	31.4		25.68- 42.8
Uric Acid *Uricase*	4.1	mg/dL	2.4 - 5.7
Calcium Serum *BAPTA*	9.2	mg/dL	8.6 - 10.0
Phosphorus *Phosphomolybdate*	4.3	mg/dL	2.5 - 4.5
Sodium *ISE-Indirect*	142.6	mmol/L	136 - 145
Potassium *ISE-Indirect*	4.6	mmol/L	3.5 - 5.1
Chloride *ISE-Indirect*	**108.2**	mmol/L	98 - 107

Interpretation:
Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases."**eGFR test is applicable for patients aged 18 years or more."**

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LABORATORY REPORT

Redcliffe labs

Patient NAME	: **Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29930269
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 02:28 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Lipid Profile

Total Cholesterol *Cholesterol Oxidase and Peroxidase*	133	mg/dL	<200
Triglycerides *4-Aminophenazone & 4-Chlorophenol*	51.5	mg/dL	<150
HDL Cholesterol *CHER & CHOD*	**37.1**	mg/dL	>40
Non HDL Cholesterol *Calculated*	95.9	mg/dL	<130
LDL Cholesterol *Cholesterol Esterase and Cholesterol Oxidase*	85.6	mg/dL	<100
V.L.D.L Cholesterol *Calculated*	10.3	mg/dL	< 30
Chol/HDL Ratio *Calculated*	3.58	Ratio	3.0 - 5.0
HDL/ LDL Ratio *Calculated*	**0.43**	Ratio	0.5 - 3.0
LDL/HDL Ratio *Calculated*	**2.31**	%	2.5 - 3.5

Interpretation:
Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Risk Category	A. CAD with > 1 feature of high risk group
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence

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High Risk	of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.

Preeti

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Test Description	Value(s)	Unit(s)	Reference Range
Iron Studies			
Iron *FerroZine*	78.8	µg/dL	33 - 193
TIBC,(Total Iron Binding Capacity) *Calculated*	302.8	µg/dL	228 – 428
UIBC *Ferene*	224	µg/dL	135 - 392
Transferrin Saturation *Calculated*	26.02	%	16 - 45

Interpretation:
Increased levels due to iron ingestion or ineffective erythropoiesis.Decreased levels due to infection, inflammation, malignancy, menstruation and Fe deficiency.Needs to be taken into consideration with TIBC. Transferrin Saturation:- Low level Transferrin Saturation can indicate iron deficiency, erythropoiesis, infection, or inflammation. High level Transferrin Saturation can indicate recent ingestion of dietary iron,ineffective erythropoiesis,haemochromatosis or liver disease.High TIBC, UIBC, or transferrin usually indicates iron deficiency, but they are also increased in pregnancy and with the use of oral contraceptives. Low TIBC, UIBC, or transferrin may occur if someone has:Hemochromatosis, Certain types of anemia due to accumulated iron,Malnutrition,kidney disease that causes a loss of protein in urine.

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Test Description	Value(s)	Unit(s)	Reference Range

High Sensitivity C-Reactive Protein (Hs-CRP)

HIGHLY SENSITIVE C-REACTIVE PROTEIN (hs-CRP) *immunoturbidimetric*	0.3	mg/L	<1.00

Interpretation:

Cardio CRP In mg/L	Cardiovascular Risk
<1	Low
1-3	Average
3-10	High
>10	Persistent elevation may represent Non cardiovascular inflammation

Note: To assess vascular risk, it is recommended to test hsCRP levels 2 or more weeks apart and calculate the average

Comments:
High sensitivity C Reactive Protein (hsCRP) significantly improves cardiovascular risk assessment as it is a strongest predictor of future coronary events. It reveals the risk of future Myocardial infarction and Stroke among healthy men and women, independent of traditional risk factors. It identifies patients at risk of first Myocardial infarction even with low to moderate lipid levels. The risk of recurrent cardiovascular events also correlates well with hsCRP levels. It is a powerful independent risk determinant in the prediction of incident Diabetes.

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Test Description	Value(s)	Unit(s)	Reference Range

Vitamin B12 / Cyanocobalamin

Vitamin - B12 *CLIA*	**166**	pg/mL	Normal Range:180-914 Indeterminate Range :145-180 Deficient Range:<145

Interpretation:
Low Values are a sign of a vitamin B12 deficiency. People with this deficiency are likely to have or develop symptoms.
Causes of vitamin B12 deficiency include:Not enough vitamin B12 in diet (rare except with a strict vegetarian diet), Diseases that cause malabsorption (for example, celiac disease and Crohn's disease), Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy. Increased vitamin B12 levels are uncommon. Usually excess vitamin B12 is removed in the urine. Conditions that can increase B12 levels include: Liver disease (such as cirrhosis or hepatitis), Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

Vitamin B12: Low Levels can cause malabsorption, Lack of intrinsic factor, Above normal heat production (for example, with hyperthyroidism), Pregnancy.High Level Liver disease, Myeloproliferative disorders (for example, polycythemia vera and chronic myelocytic leukemia).

1. Out of 140 healthy indian population, 91% of Vitamin B 12 concentrations was at lower level: 59.00 pg/ml and upper level: 700.00 pg/ml

"Patients on Biotin supplement may have interference in some immunoassays. Ref: Arch Pathol Lab Med—Vol 141, November 2017. With individuals taking high dose Biotin (more than 5 mg per day) supplements, at least 8-hour wait time before blood draw is recommended."

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Test Description	Value(s)	Unit(s)	Reference Range
Vitamin D 25 Hydroxy			
Vitamin D 25 - Hydroxy *CLIA*	**11.9**	ng/mL	Deficiency : < 10 ng/mL Insufficient : 10-30 ng/mL Sufficient : 30-100 ng/mL Hypervitaminosis : > 100 ng/mL

Interpretation:
25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

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Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 29930269
Referred BY	: Self	Sample Type	: Serum
Sample Collected	: Jan 20, 2026, 08:15 AM	Report Date	: Jan 20, 2026, 02:28 PM.

Test Description	Value(s)	Unit(s)	Reference Range

Thyroid Profile Total

Triiodothyronine (T3) *CLIA*	1.1	ng/mL	0.71 - 2.01
Total Thyroxine (T4) *CLIA*	7	µg/dL	5.1 - 14.1
Thyroid Stimulating Hormone (Ultrasensitive) *CLIA*	2.2	µIU/mL	0.27 - 4.20

Interpretation:

Pregnancy	Refrence Range TSH
1st Trimester	0 .1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Clinical Use:
1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance
Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.
2. Secondary/Tertiary Hypothyroidism: Low TSH levels.
3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Nomral	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

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Test Description	Value(s)	Unit(s)	Reference Range

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LABORATORY REPORT

Redcliffe labs

Patient NAME	**: Ms Sonal Mittal**		
DOB/Age/Gender	: 23 Y/Female	Report STATUS	: Final Report
Patient ID / UHID	: 15214055/RCL15051215	Barcode NO	: 30880395
Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Jan 20, 2026, 08:20 AM	Report Date	: Jan 20, 2026, 05:21 PM.

Urine Routine and Microscopic Examination

Test Description	Value(s)	Unit(s)	Reference Range
Physical Examination			
Volume *Visual*	20	mL	-
Colour *Visual*	Yellow	-	Pale yellow
Transparency *Visual*	**Hazy**	-	Clear
Deposit *Visual*	**Present**	-	Absent
Chemical Examination			
Reaction (pH) *Double Indicator*	6.0		
Specific Gravity *Ion Exchange*	1.010		
Urine Glucose (sugar) *Benedicts*	Negative		
Urine Protein (Albumin) *Acid/Base colour exchange*	Negative		
Urine Ketones (Acetone) *Legals Test*	Negative		
Blood *Peroxidase Hemoglobin*	**Positive(+)**		
Leucocyte esterase *Indoxyl Ester*	Negative		
Bilirubin Urine *Coupling Reaction*	Negative		
Nitrite *Griless Test*	Negative		
Urobilinogen *Ehrlichs Test*	Normal		
Microscopic Examination			
Pus Cells (WBCs) *Wet Mount*	2-4	/hpf	0 - 5
Epithelial Cells *Wet Mount and Microscopy*	1-2	/hpf	0 - 4
Red blood Cells *Wet Mount and Microscopy*	10-12	/hpf	Absent
Crystals *Wet Mount and Microscopy*	Absent	-	Absent
Cast	Absent	-	Absent
Yeast Cells *Wet Mount and Microscopy*	Absent	-	Absent
Amorphous deposits	Absent	-	Absent

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Referred BY	: Self	Sample Type	: Spot Urine
Sample Collected	: Jan 20, 2026, 08:20 AM	Report Date	: Jan 20, 2026, 05:21 PM.

Test Description	Value(s)	Unit(s)	Reference Range
Bacteria *Wet Mount and Microscopy*	Absent	-	Absent
Protozoa *Wet Mount and Microscopy*	Absent	-	Absent

Interpretation:

URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications.

Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise.

Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders.

Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection.

Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection.

pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine.

Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus.

Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine.

Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.

*** End Of Report ***

Preeti

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LABORATORY REPORT

Redcliffe labs

Bio-Rad CDM System
VII Inst. #1. SN-15428

PATIENT REPORT
V2TURBO_A1c_2.0

Patient Data

Sample ID: 29157878
Patient ID:
Name:
Physician:
Sex:
DOB:

Comments:

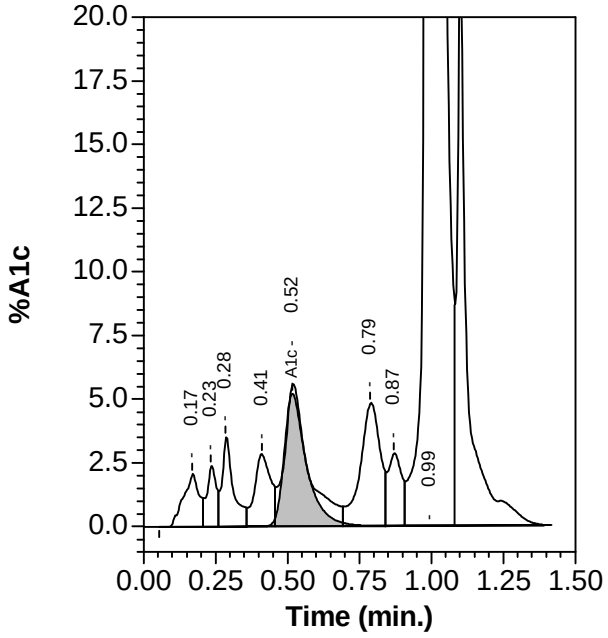
Analysis Data

Analysis Performed: 20/JAN/2026 14:03:32
Injection Number: 4302
Run Number: 230
Rack ID: 0006
Tube Number: 1
Report Generated: 20/JAN/2026 14:07:06
Operator ID:

Peak Name	NGSP %	Area %	Retention Time (min)	Peak Area
A1a	---	1.2	0.167	13687
A1b	---	0.8	0.232	9530
F	---	1.4	0.284	16646
LA1c	---	1.6	0.410	19056
A1c	5.2	---	0.517	49028
P3	---	3.4	0.787	40232
P4	---	1.3	0.868	15916
Ao	---	86.1	0.993	1017570

Total Area: 1,181,667

HbA1c (NGSP) = 5.2 %



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2. It is to be presumed that the tests performed pertain to the specimen/sample attributed to the Customer's name or identification. It is presumed that the verification particulars have been cleared out by the customer or his/her representation at the point of generation of said specimen / sample. It is hereby clarified that the reports furnished are restricted solely to the given specimen only.

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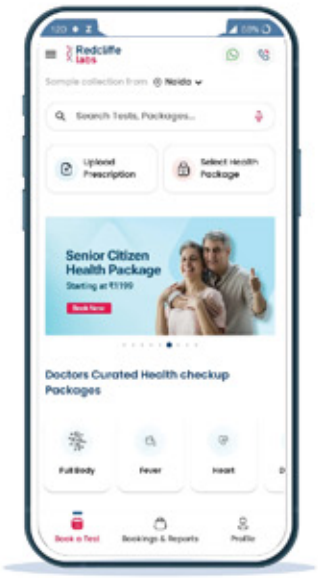

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